CareJiyo

Administrator manual

Configuration, access and controlled maintenance

ENGLISH

For authorised clinic administrators. Keep this volume separate from the everyday user manuals.

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CareJiyo Learning

Contents

Select a heading to jump to the section. PDF bookmarks provide the same navigation.

Administrator responsibilities and setup order 4

1.1 Who should use this manual . . . 5

1.2 Company, facilities, areas and initial checks . . . 6

People, roles and catchment access 7

2.1 Create, change or disable an account . . . 8

2.2 Access & roles, staff linkage and scope . . . 9

Master data, services and pricing 10

3.1 Maintain approved dropdown values and equipment . . . 11

3.2 Services, pricing engines and rules . . . 12

3.3 Import a price list and maintain packages . . . 13

Forms and clinical configuration 14

4.1 Form Studio and required fields . . . 15

4.2 Clinical form templates and medical catalogues . . . 16

4.3 Menu layout and clinic settings . . . 17

Connections, communications and AI 18

5.1 Channels, phone and website enquiries . . . 19

5.2 Reply templates, reminders and family access . . . 20

5.3 AI assistance and what remains a human decision . . . 21

Records, compliance and controlled corrections 22

6.1 Audit Log, consent checks and data lifecycle . . . 23

6.2 Compliance Register and evidence . . . 24

6.3 Rights requests, calendars and retention holds . . . 25

6.4 Processing destinations and authority integrations . . . 26

6.5 Terminology, EMR export, readiness and EVV . . . 27

Reporting and learning administration 28

7.1 Analytics sources, datasets and dashboards . . . 29

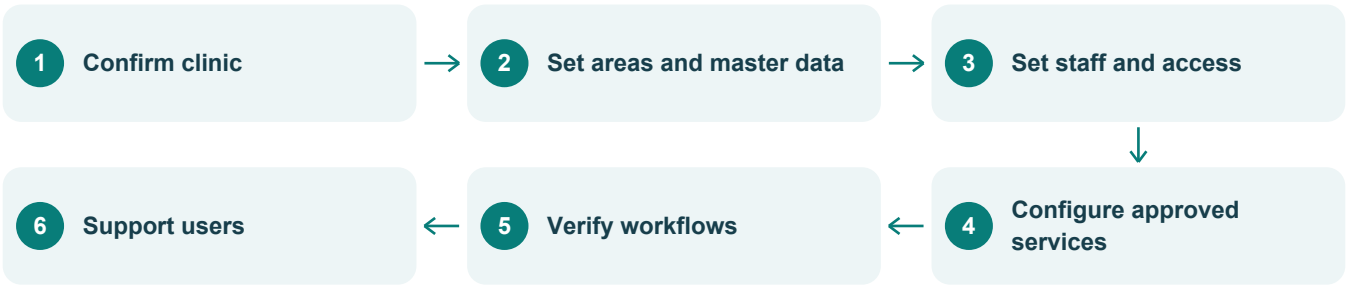
7.2 Maintain training and interpret learner progress . . . 30

7.3 Routine review and safe troubleshooting . . . 31

CHAPTER 01

Administrator responsibilities and setup order

The normal process



In this chapter

Who should use this manual

Company, facilities, areas and initial checks

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

1.1 Who should use this manual

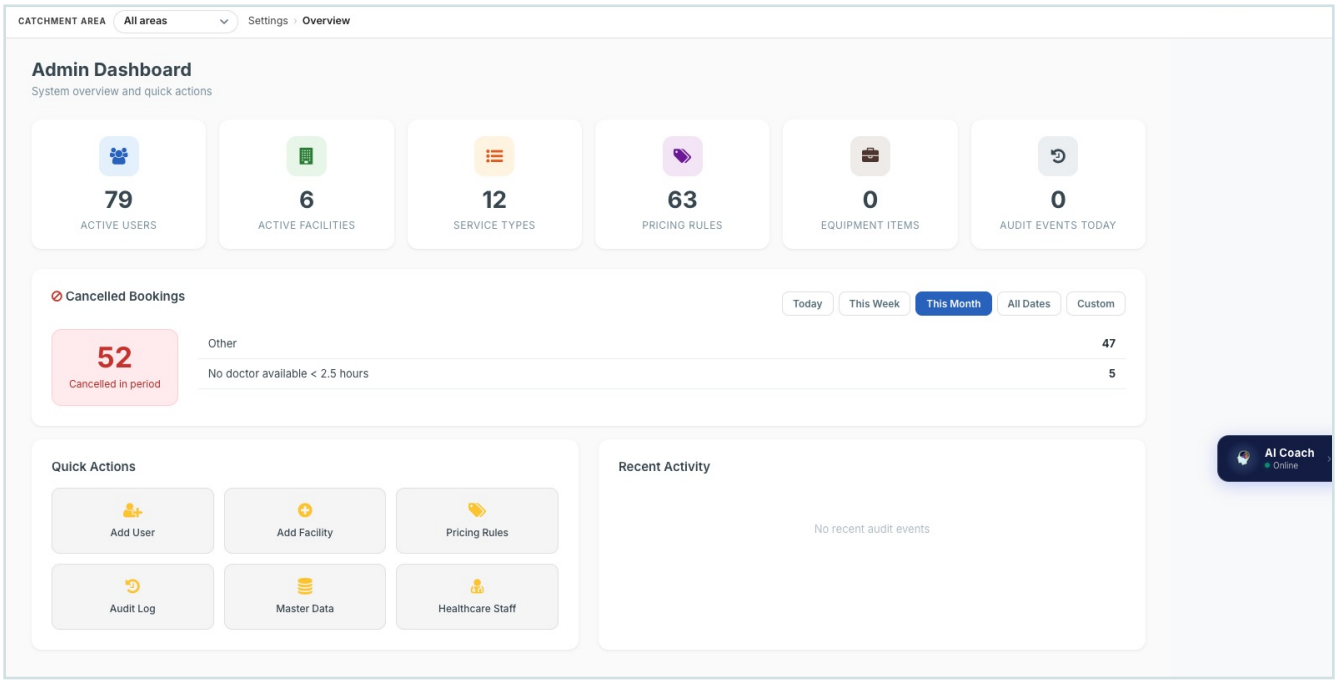


Figure 1. Administrator overview: common setup areas and recent activity.

This manual is for the clinic owner, authorised administrators and designated specialists who maintain configuration. It is separate from the everyday user manual because configuration can change what many people see and do. Keep distribution limited to the appropriate staff.

An administrator maintains the system; a clinical lead approves care content; a finance lead approves tariffs and financial procedures. Technical access does not replace those responsibilities. Ask the relevant owner to approve the business rule before changing it.

Use Settings > Overview to reach common setup areas. People controls accounts and staff; Master data controls approved reference values; Connections controls external services; Records provides history and lifecycle tools. Your company, Clinic setup and Activity history support the clinic workspace.

Before a change, note the affected company, role, catchment and existing value. Make a focused change, save it, then inspect the actual screen as the affected role. A menu being visible does not prove the user can read or change its records. Keep a brief record of the reason and result.

1.2 Company, facilities, areas and initial checks

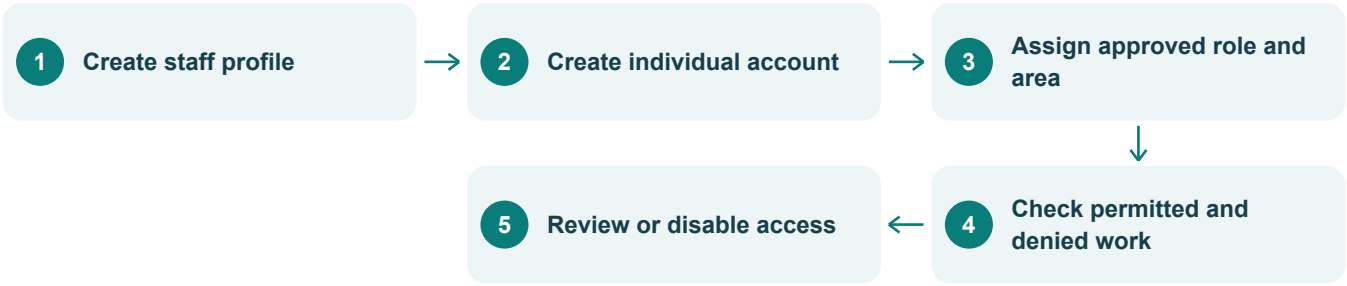
1. Confirm the clinic identity and company information through Your company or the available company screen. Keep display name, contact details and branding consistent.
2. Define facilities and geographic catchments. Check the association between facilities, clients and staff before importing or assigning work.
3. Confirm timezone, operating hours and public holidays. Review a sample appointment near a day boundary to catch timezone mistakes.
4. Create approved reference data, service categories and pricing before asking staff to quote.
5. Create staff profiles and individual accounts with the appropriate roles and employee links.
6. Verify a small complete workflow: enquiry, client, booking, assignment, visit completion and invoice review. Use approved practice records and avoid unintended external messages or financial transactions.

Clinic setup may show readiness steps or incomplete items. Treat each as a prompt to inspect the actual underlying configuration, not as a substitute for checking business results. Platform-level Customers or tenancy screens, if visible to a platform operator, are outside routine clinic administration; do not use them to manage a normal client/patient.

CHAPTER 02

People, roles and catchment access

The normal process



In this chapter

Create, change or disable an account

Access & roles, staff linkage and scope

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

2.1 Create, change or disable an account

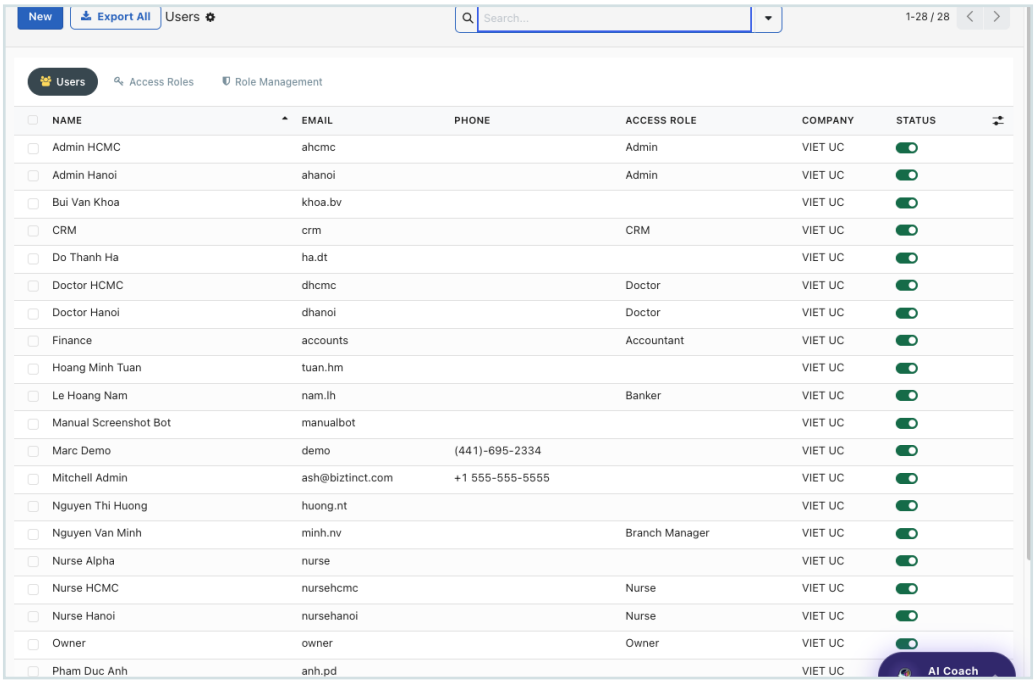


Figure 2. Users & Roles: locate the individual account before changing access.

Use Settings > People > Users & Roles. Search for the person first so one employee does not acquire duplicate accounts.

1. Create the named person with a clear individual login and the approved role. Follow the available account-activation or password process; do not put passwords in shared manuals.
2. Set company, facility and catchment as required. Link the employee record for nurse/doctor work so assignments match the login.
3. Save and ask the person to sign in. Check their starting screen and role-specific tasks.
4. Test one permitted action and one action they should not be allowed to perform. Check record visibility as well as menu visibility.
5. When someone leaves or changes duties, update the role or deactivate the account through the supported control. Reassign outstanding work and retain the history under the clinic's policy.

A login is the identity used to sign in. The employee record holds work-related details. The business role grants a defined set of functions. The catchment limits geography. All four must agree; fixing one does not automatically fix the others.

2.2 Access & roles, staff linkage and scope

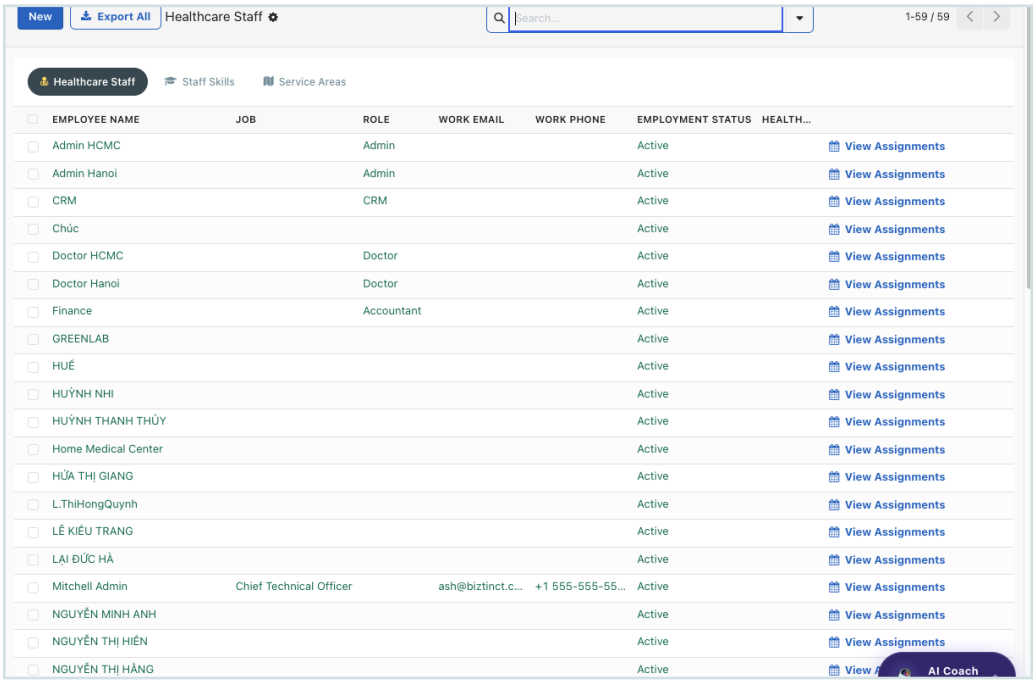


Figure 3. Healthcare Staff: maintain the work profile that supports assignment.

Access & roles describes the business roles and the screens they open. Use the role intended for the job rather than adding broad groups until a screen opens. Owner/Admin access should remain separate from everyday work.

Operations Managers must be limited to their assigned catchments. Nurses and doctors need the correct clinical role, staff link and permitted area; visit actions also depend on assignment. Finance and CRM access should match their own work. A catchment dropdown is a convenience for selecting visible data, not a substitute for server-side restrictions.

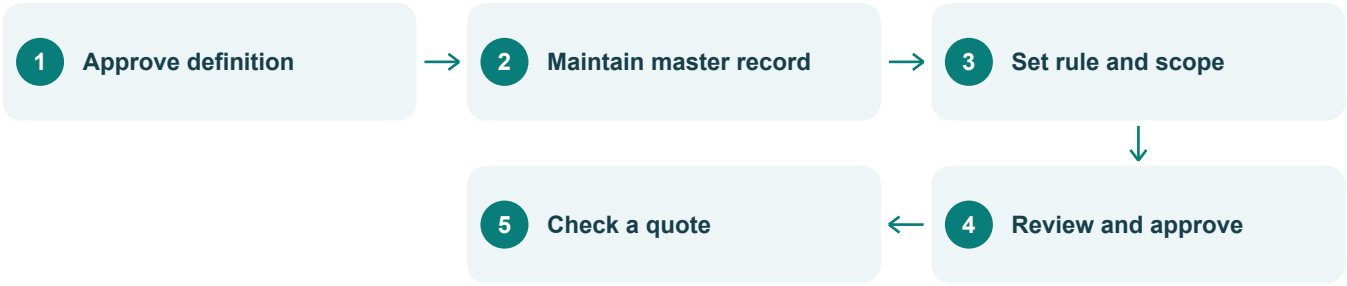
When someone cannot see a record, check: active account; company; role; employee link; employee availability; facility/catchment; record's own catchment; and current assignment. Use a known permitted record and a known out-of-scope record to distinguish a filter issue from an access issue.

Healthcare Staff holds the people who can be assigned: role, skills, facilities, service areas and other work details. Maintain these with the staffing manager. Disabling a login does not automatically resolve that person's future assignments; review the schedule separately.

CHAPTER 03

Master data, services and pricing

The normal process



In this chapter

Maintain approved dropdown values and equipment

Services, pricing engines and rules

Import a price list and maintain packages

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

3.1 Maintain approved dropdown values and equipment

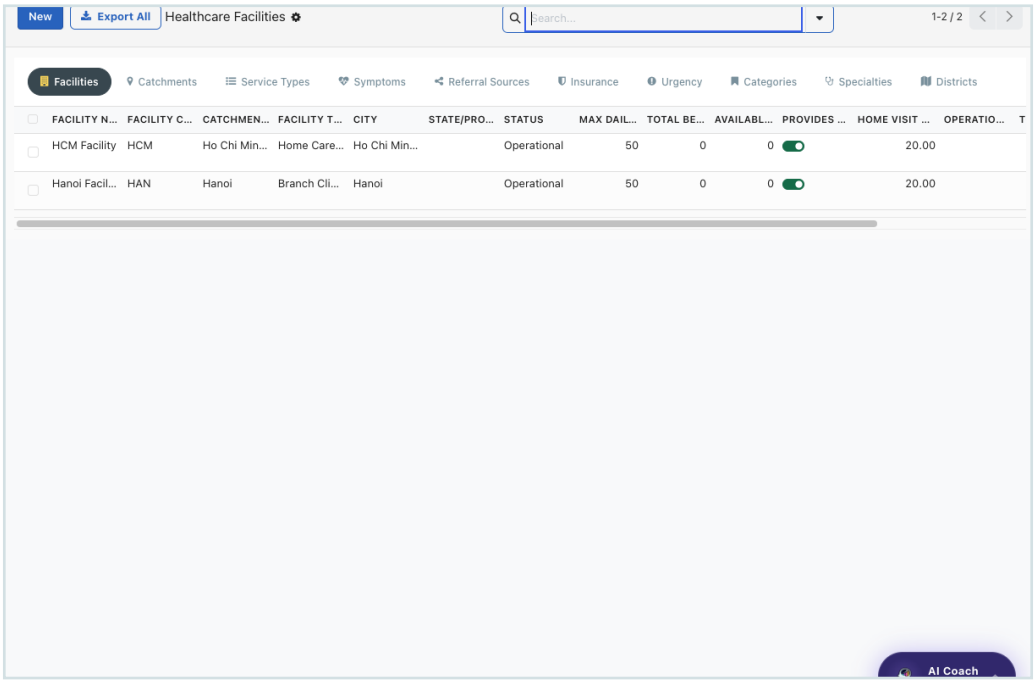


Figure 4. Master-data navigator: select the correct reference list.

Master data supplies values used across forms: facilities, catchments, service types, symptoms, referral sources, insurance, urgency, patient categories, specialties and districts where available. Different lists can be reached through the Facilities/master-data navigator and related tabs.

1. Search the existing list, including inactive entries if permitted, before adding a value.
2. Create a clear name and the required code or relationships. Use consistent spelling and avoid near-duplicate labels.
3. Save, then open the actual business form and confirm the new value is available in the correct dropdown.
4. For an obsolete value, prefer the supported archive/deactivate process when appropriate. Consider records already referring to it before deletion or renaming.

Equipment maintains portable items that can be attached to work. Record identity, availability and relevant service details, then verify selection on the booking. Public Holidays records dates used by scheduling and pricing; check the date, location/scope and applicability. A holiday entry can affect calculations, so verify both an ordinary day and the holiday after a change.

3.2 Services, pricing engines and rules

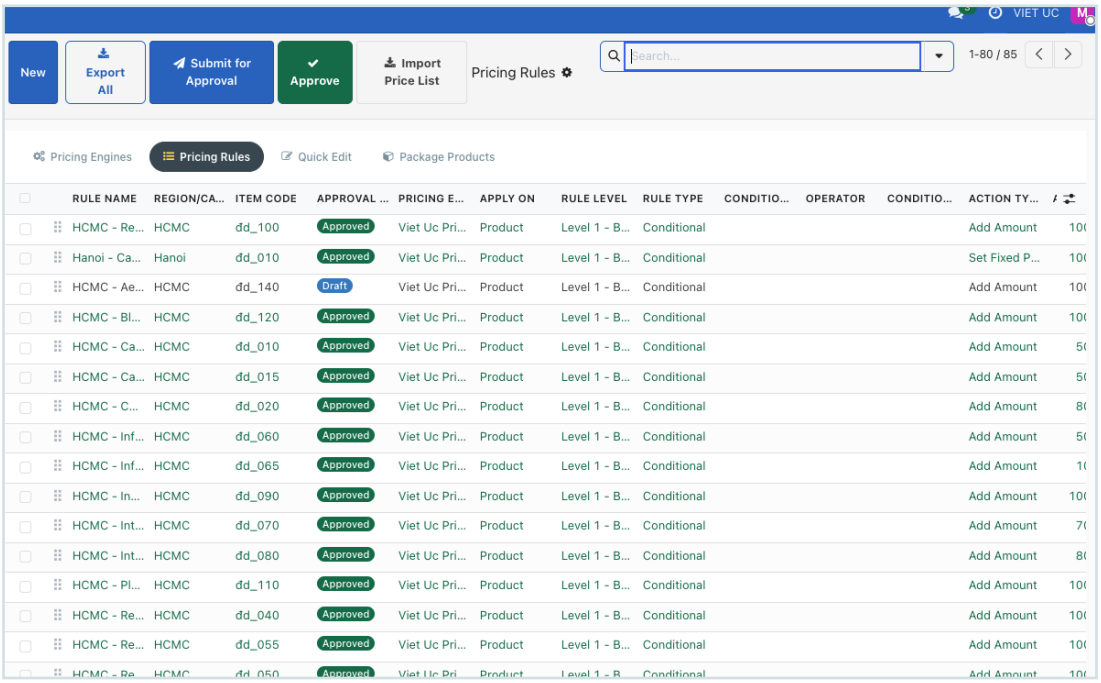


Figure 5. Pricing: distinguish rule editing, approval and price-list import.

Open Settings > Master data > Pricing. The area includes service/product information and pricing views such as engines, rules, quick edit and packages, depending on the selected tab.

A service/product identifies what is sold. A pricing engine groups calculation behaviour. A rule specifies when a price or adjustment applies. Rules can distinguish catchment, service, quantity or duration, distance, time, urgency and other configured conditions. Sequence and rule level can affect calculation order.

1. Confirm the approved tariff and effective scope with the finance owner.
2. Locate the existing service or rule by code and catchment. Check whether the change belongs to the base service price or an adjustment rule.
3. Update the relevant conditions and action, such as a fixed amount or surcharge. Check currency, unit, percentage and quantity interpretation.
4. Use Submit for Approval and Approve through the authorised process where those controls apply. A saved draft rule is not necessarily an approved active tariff.
5. Create or recalculate a suitable practice quote and inspect its breakdown. Check a case that should match the rule and one that should not.

Never judge success by the rule screen alone. The final service, travel, time and discount breakdown must agree with the intended calculation.

3.3 Import a price list and maintain packages

Import Price List opens the supported import wizard. Read its available template, matching options and import summary. A spreadsheet row must match the intended service or rule; identical names across different areas are not enough.

Before importing, retain an authorised copy of the current values and identify which codes and catchments will change. Check headings, amounts, units and duplicates in the proposed file. Import through the wizard, review errors and counts, then compare representative records against the approved file. Quick Edit changes records more rapidly but still needs the same review.

Check examples across ordinary weekdays, weekends, holidays, evening/night time, travel boundaries, extra hours, discounts and added services when those rules apply. Confirm no unintended duplicate rules are contributing twice. Old invoices and quotations may retain historical values; do not assume a tariff update should rewrite historical transactions.

For packages, define the correct included services, entitlement and validity. Check sale, payment, use on an eligible visit and remaining entitlement. Keep product definitions separate from a client's purchased package and usage history.

CHAPTER 04

Forms and clinical configuration

The normal process



In this chapter

Form Studio and required fields

Clinical form templates and medical catalogues

Menu layout and clinic settings

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

4.1 Form Studio and required fields

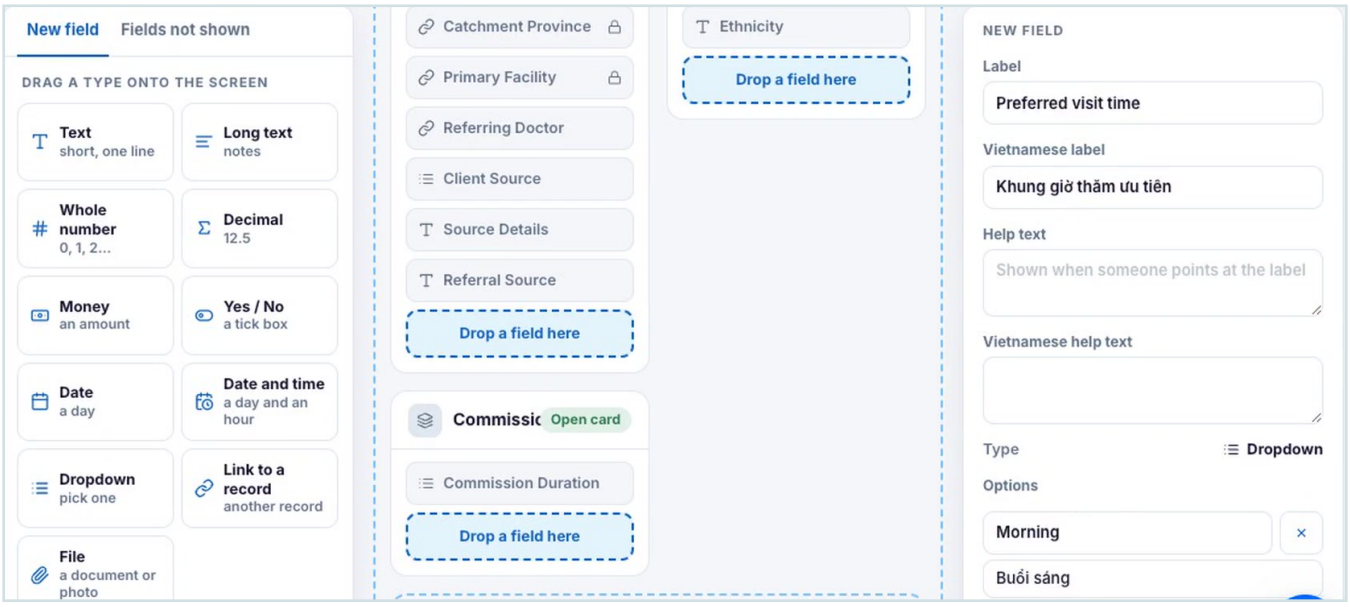


Figure 6. Form Studio: define a field and review its type, label and options.

Form Studio customises supported business forms and lists, including Booking, Client, Contact and Incident. Field Requirements controls when information is mandatory. These are different tools: one changes structure or placement; the other controls completion requirements.

1. Identify the exact form/model and intended audience. Write down the business purpose, field type and label before editing.
2. Open Form Studio, select the supported form and add or place the field using its editor. Choose an appropriate text, number, selection, date or boolean type.
3. Review visibility, placement and available publishing controls. Preview the form before applying the layout.
4. If the field must be mandatory, configure the correct role and state conditions in Field Requirements. Avoid making it required before staff can reasonably know the answer.
5. Check a new record, an existing record and the relevant mobile or alternate screen. Confirm data is saved and the field does not block unrelated roles.

Changing a dropdown label or removing a field can affect reporting and existing data. Keep the meaning stable and plan the correction with the business owner. Do not use a new custom field as a substitute for an existing standard field with the same purpose.

4.2 Clinical form templates and medical catalogues

Form Templates defines clinical assessments. A template contains questions, answer types, required flags and optional scoring bands. Publish makes an approved version available; Retire removes it from future use through the supported lifecycle; New Version supports controlled change. Preserve the meaning of completed assessments.

1. Have the clinical owner agree the wording and scoring.
2. Create the draft questions and options, including units and required conditions.
3. Preview a complete example and a missing-answer example. Check the calculated band and any extracted observation mappings.
4. Publish the approved version and verify it appears for the intended visit workflow.

Observation Types defines structured measurement types, units and terminology links. Thresholds are clinical configuration and must be approved by the clinical lead. Medication Catalog defines medicines referenced by orders; maintaining a catalogue does not prescribe a treatment. Monitoring Devices links a device and client, with active, suspended and retired states. Check the identity, expected source and latest readings before treating incoming values as reliable.

Use Suspend or Retire through the supported device action when appropriate. Investigate data-source errors separately from an alert about a client's condition.

4.3 Menu layout and clinic settings

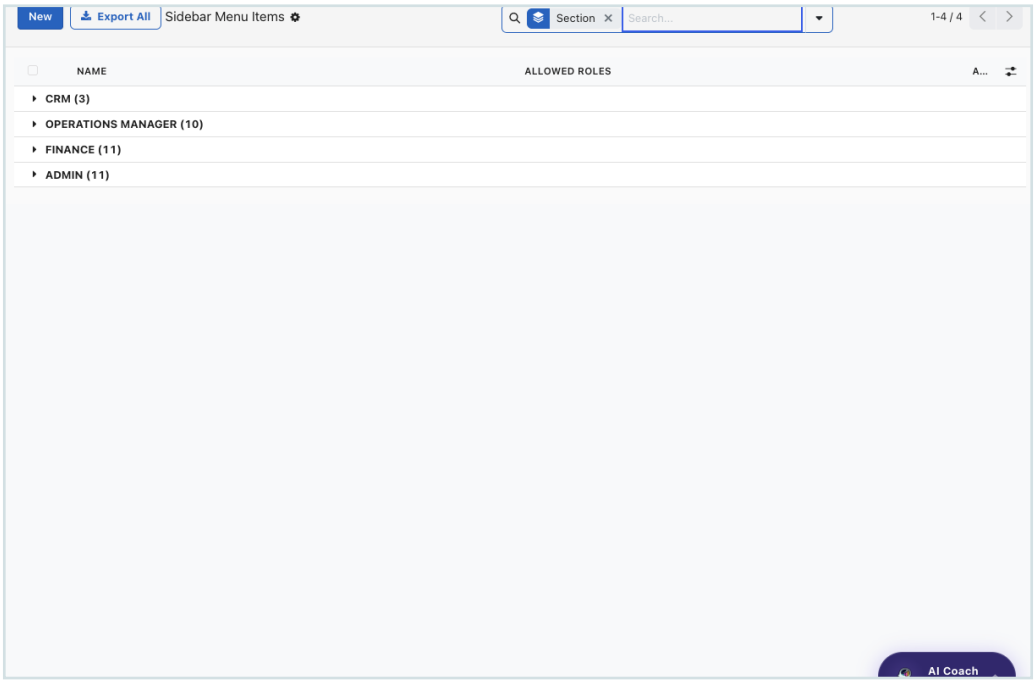


Figure 7. Menu configuration: check the destination action and grouping.

Settings > Menu controls navigation entries, sections and grouping. A heading may group several screens; an item points to a particular action. Renaming or moving an item changes navigation, not the underlying model permissions.

Before changing a menu, identify its existing action and intended roles. Keep the structure understandable: area, tab, then screen. Check that a renamed item still opens the correct screen and that child screens remain reachable. Review both English and Vietnamese labels where maintained. Do not expose an administrator screen to ordinary users as a workaround for an access error.

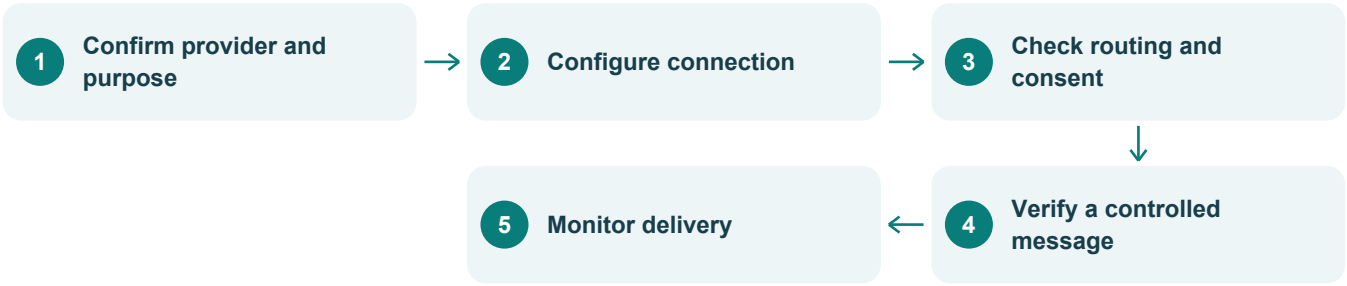
Settings contains company-wide options such as maps, billing and integrations, depending on installed modules. Change only the option belonging to the intended company and workflow. After a change, check a representative client or booking rather than relying on a successful Save message.

About and Activity history help identify the workspace and review changes. Record the reason for a configuration change so a later administrator can understand it.

CHAPTER 05

Connections, communications and AI

The normal process



In this chapter

Channels, phone and website enquiries

Reply templates, reminders and family access

AI assistance and what remains a human decision

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

5.1 Channels, phone and website enquiries

Settings > Connections contains Channels (setup), Channel Go-Live, Website Connector, Phone system and Extensions where enabled. These screens configure services; seeing a menu does not mean a provider is connected.

For each channel, identify the clinic account, permitted purpose, responsible owner and routing destination. Follow the provider-specific connection workflow, store credentials only in the designated fields, and check connection status. Use Channel Go-Live to review the available readiness checks before actual use.

Phone system and Extensions map the provider account and staff extensions. Verify incoming-call routing, call history and permitted recording access with the phone owner. Website Connector configures how website enquiries enter the clinic; confirm the intended site, destination and attribution fields. Google Ads application configures the provider application separately from the connected advertising account shown to CRM.

After setup, use an authorised example message or enquiry and trace it through Unrouted Contacts, Channel messages, Channel audit and the resulting contact. Confirm errors and retries do not create duplicates. Live-channel operation requires the actual provider accounts and approved connection; a saved configuration alone is not evidence of live delivery.

5.2 Reply templates, reminders and family access

Reply Templates provides reusable wording; Watchlist phrases supports configured recognition in channel work. Have the responsible team approve wording and language. Check placeholders and a real preview so the wrong client name or date is not sent.

Visit Messaging maintains outgoing confirmation/reminder records and their outcomes. Its enabled setting and dry-run mode determine whether messages are actually sent. A dry-run record is a simulation, not delivery. The implemented routing can use configured Zalo ZNS, email or staff-call follow-up according to availability and outcome; inspect the record rather than assuming every channel was used.

Family links, portal access and self-booking invitations are tied to a client or visit. Verify recipient identity, consent, scope and validity. Use the supported revoke or token-rotation action when access should end or a link is exposed. Do not paste private links into broad group conversations.

Review failed delivery, fallback and duplicate-prevention behaviour with the operational owner. Changing a template does not correct a message already sent; record the correction and communicate it through the approved process.

5.3 AI assistance and what remains a human decision

AI functions have different purposes. Care Command may suggest a reply. Voice Notes may produce a transcript. Coding Review may suggest a medical code. Coaching may suggest a summary or action. The learning coach retrieves published instructions. Do not assume enabling one feature approves every type of data processing.

Use the existing approved provider configuration and processing-destination controls. Identify what information each feature sends, who may use it and what consent or approval is required. Verify the feature's enabled state and failure behaviour before making it available to staff.

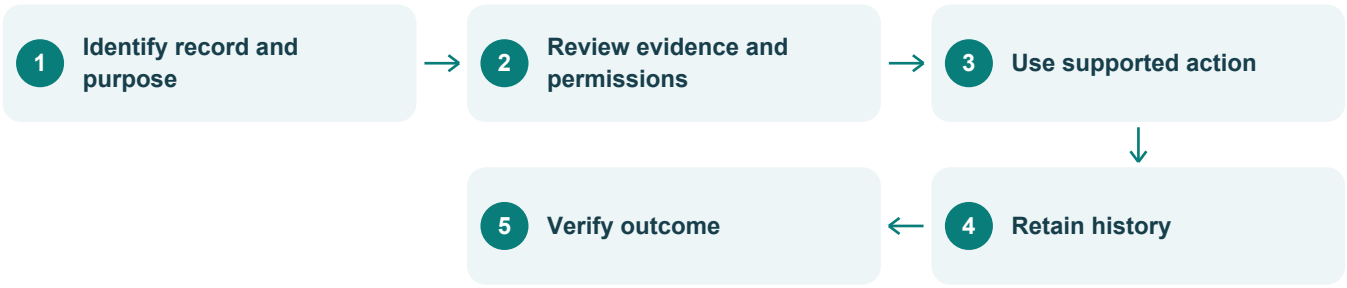
The user must review a reply before sending, a transcript before saving as the clinical note, a proposed code before approval and a coaching recommendation before adopting it. The learning coach cannot perform business actions or replace clinical judgement.

When an AI feature is unavailable, use the supported manual workflow. Do not copy client records to an unrelated consumer AI service to bypass a configuration or consent restriction. Review logs only with the appropriate permissions and avoid exporting sensitive payloads unnecessarily.

CHAPTER 06

Records, compliance and controlled corrections

The normal process



In this chapter

Audit Log, consent checks and data lifecycle

Compliance Register and evidence

Rights requests, calendars and retention holds

Processing destinations and authority integrations

Terminology, EMR export, readiness and EVV

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

6.1 Audit Log, consent checks and data lifecycle

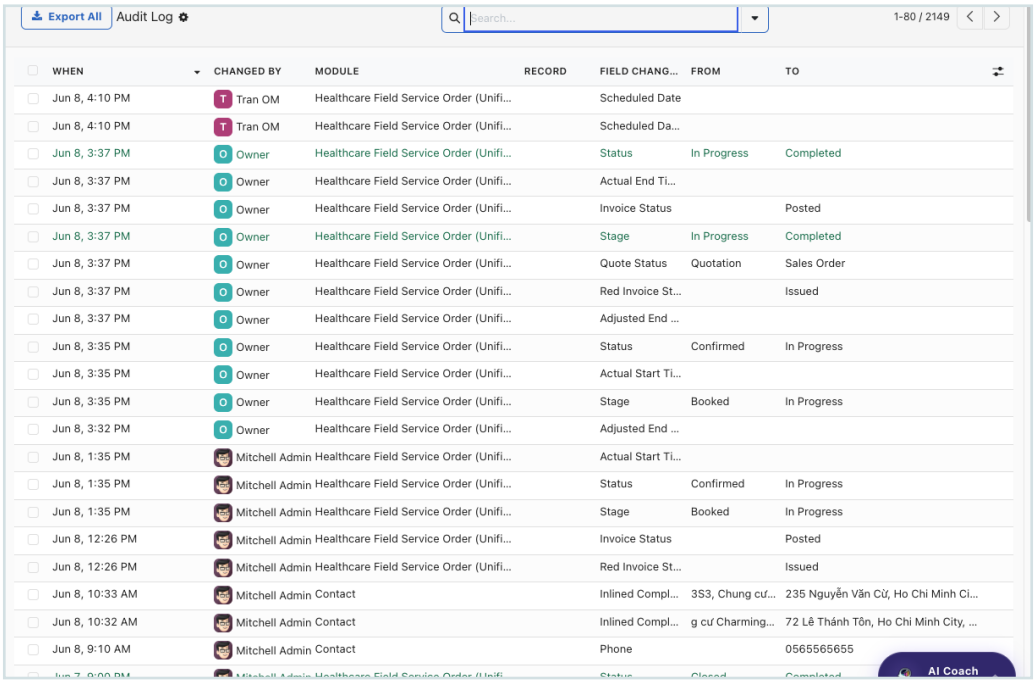


Figure 8. Audit history: narrow by record and date, then inspect the event.

Settings > Records > Audit Log helps trace who changed a record and when. Use the model, reference, person and date filters to narrow the search. Review the event and linked record together; a log entry must be interpreted in context.

Consent Check Log records available permission checks. Use it when investigating why a sharing or processing action was allowed or blocked. Do not edit the consent simply to make a past failed action appear valid.

Data Lifecycle records supported archive, deletion and restoration activity. Archiving removes a record from ordinary active lists; it is not necessarily permanent deletion. A deletion action can have stricter permissions and requirements. Read the impact, reason and any retention hold before proceeding. Use restore only where the application offers a supported recovery route.

For important corrections, retain the original reference, reason, responsible person and resulting record. A final clinical record should use its permitted addendum process. Review the actual corrected output after the action, not just the absence of an error message.

6.2 Compliance Register and evidence

Compliance Register organises requirements and their supporting evidence. Technical verification, operational/manual evidence and authority approval are separate dimensions. A working software feature is not, by itself, proof that an external authority has approved it.

Evidence Artifacts records supporting documents and their scope. Enter a meaningful description, the relevant requirement, date and responsible owner. Approve after review should only be used after the authorised reviewer has checked the actual evidence. Preserve reviewed records and use the available correction process when information changes.

1. Open the requirement and read the expected outcome.
2. Check which evidence relates to the current company, environment and period.
3. Add the relevant proof through the permitted evidence action.
4. Review the separate statuses and unresolved items.
5. Export the register only for an authorised audience and check what the export includes.

The register is a work and evidence tool. It does not automatically establish legal compliance or national-system acceptance. Assign an owner to unresolved requirements and track the next action.

6.3 Rights requests, calendars and retention holds

Rights Requests supports requests concerning personal information. Record the requester, subject, request type, received date, responsible owner and verified identity information through the permitted fields. Handle identity evidence through the clinic's approved process.

Record acknowledgement requires proof of the actual acknowledgement. Record extension notice captures an actual notice when an extension is authorised. Complete request should follow the completed outcome and its communication evidence. A computed deadline helps organise work; the responsible privacy lead must confirm the applicable requirement and any exception.

Rights Calendar maintains the relevant working/non-working-day information used by the workflow. Retention Holds prevents selected data from being removed while a legitimate hold applies. Record the scope, reason and responsible owner. Release hold only when the authorised decision permits it.

Before deleting or exporting information in response to a request, check identity, scope, clinical-record obligations and active holds with the responsible specialist. Keep a record of what was provided or corrected. Do not treat a request as permission to expose unrelated people's information.

6.4 Processing destinations and authority integrations

Processing Destinations identifies recipients or services that may receive information for an approved purpose, such as AI processing or support. Review purpose, scope and supporting approval before using Approve destination. Do not grant a broad destination merely to make one integration work.

Authority Integrations tracks the readiness of external capabilities such as certified signing or national-health connections. Record the actual provider, contract/onboarding status and evidence. A configured adapter or locally generated file is not evidence of a successful production authority connection.

Where the screen distinguishes pending, configured or approved states, preserve that distinction in operational reports. Do not enter an approval state without the required proof. Review changes with the responsible clinical, privacy or integration owner.

Secrets, access tokens and provider credentials belong only in the intended secured configuration controls. Never include them in screenshots, training examples, exported user manuals or broad chat messages.

6.5 Terminology, EMR export, readiness and EVV

Terminology contains Medical Codes, Coding Systems and Import Codes. Maintain the approved coding system and version. Check code format, descriptions and duplicates when importing. Changing a code's meaning can affect existing clinical interpretation.

EMR (VN) > Readiness shows available checks for the Vietnam export profile. Resolve missing identity, facility or clinical information through its proper source record. Export prepares the supported file/bundle and records a submission-log entry. In the current adapter, file export is distinct from live national transport; a local export is not proof that a national service received or accepted it. Record actual acknowledgement only when supported by real evidence.

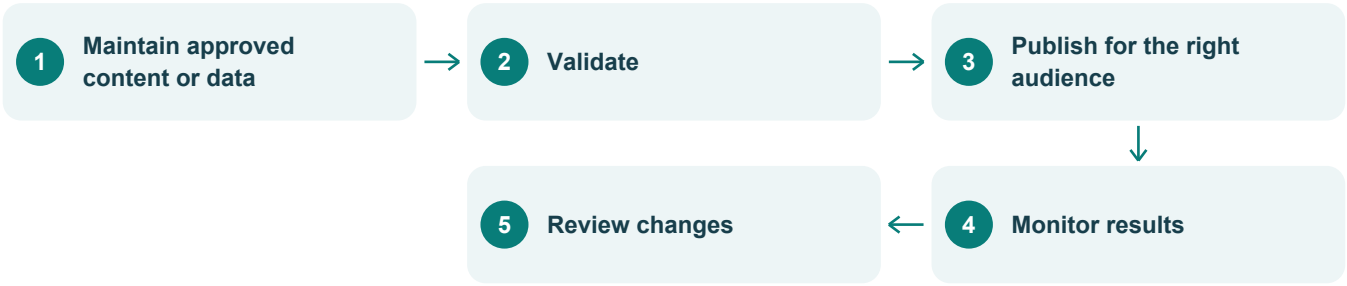
EVV Events provides visit verification history. Inspect the linked visit, event time, location and other available evidence. The event log supports traceability; it is not a screen for silently rewriting check-in history.

When investigating a missing export or verification event, preserve the reference and error, check the source data and consult the integration owner. Do not invent identity, signatures, timestamps or acknowledgement details to make a readiness check pass.

CHAPTER 07

Reporting and learning administration

The normal process



In this chapter

Analytics sources, datasets and dashboards

Maintain training and interpret learner progress

Routine review and safe troubleshooting

Read the process from step 1 onward. Your role, the record status and the enabled features determine which actions are available. Check the saved result before handing work to the next person.

7.1 Analytics sources, datasets and dashboards

Analytics > Data groups Sources, Datasets, Refresh jobs, Pipelines, Semantic model and Glossary. These are reporting configuration tools and should be managed by the reporting administrator rather than ordinary dashboard readers.

A source identifies where data comes from. A dataset defines the data made available for reporting. A pipeline prepares it. The semantic model defines business measures and relationships. The glossary explains terms. Refresh jobs bring the data up to date. A dashboard presents the selected result.

1. Confirm the approved source and access scope. Use Test Connection where available without exposing credentials.
2. Define or review the dataset and transformation. Validate before applying or publishing.
3. Check row counts, dates, measures and role restrictions using known records.
4. Publish only the intended dataset/dashboard and set recipients and schedules carefully.
5. Monitor refresh outcomes; investigate errors before users rely on stale figures.

A refreshed dataset can still contain an incorrect business definition. Compare key totals with source invoices, bookings or payments. Do not grant direct source access to solve a dashboard-permission problem.

7.2 Maintain training and interpret learner progress

Learn contains two related experiences. Task guides and videos uses the curated task library, role paths and guide-grounded coach. Training uses lessons and stations with its own learning records. Training admin includes Lessons, Stations, Learner progress, Learning events and Training wording where available.

Maintain a named owner for each learning topic. When a workflow changes, check the written steps, screenshots, captions and video chapters together. Retire or update outdated instructions rather than adding contradictory notes. Review the learner-facing wording in both languages.

Use learner progress to identify people who may need help. Reviewed, watched or quiz-completed flags do not prove clinical competence or grant application permissions. Combine them with supervised practice and the clinic's role assessment.

The curated task/video catalogue is maintained through the application's content release process, rather than by editing an arbitrary lesson record. Request a coordinated update for its guides, video files and role mappings. Keep videos within their intended access and provide administrator manuals only to the appropriate audience.

7.3 Routine review and safe troubleshooting

Daily: check unresolved delivery failures, failed refresh jobs, important access reports, pending cash and operational exceptions with the responsible teams. Weekly or at the clinic's agreed interval: review leavers and changed roles, expiring consent, upcoming holidays, outdated training and unresolved compliance evidence.

When a problem is reported, record the user, role, company, catchment, screen, reference, time and exact message. Reproduce with the least privilege needed. Separate missing data, wrong filters, wrong assignment, wrong permissions, stale data and a genuine software defect.

After a fix, check the original case and a neighbouring case that should remain unchanged. For an access change, confirm both permitted and prohibited work. For pricing, check a matching and non-matching quote. For a form, check existing records as well as new ones. For integration, verify the external outcome as well as the local status.

Keep regular users focused on their tasks. Share the user manual and relevant guides; keep this administrator manual in the designated restricted location. Escalate code/deployment issues to the technical operator with the evidence needed to investigate.